1801 Pennsylvania Avenue NW. 202 378 2025
Suite 900
Washington, D.C. 20006

June 16, 2025

Via Electronic Submission

Dr. Mehmet Oz, Administrator
Centers for Medicare & Medicaid Services
Attention: CMS-0042-NC.
P.O. Box 8013
Baltimore, MD 21244-8013.

Re: Request for Information; Health Technology Ecosystem (CMS-0042-NC)

Dear Dr. Oz:

Abbott welcomes the opportunity to comment on the Centers for Medicare & Medicaid Services (CMS) and Office of the National Coordinator for Health Information Technology (ONC) request for information (RFI) on the Health Technology Ecosystem.

Abbott is committed to helping people live their best possible life through the power of health. For more than 135 years, we've brought new products and technologies to the world – in diagnostics, medical devices, nutrition, and branded generic pharmaceuticals – that create more possibilities for more people at all stages of life. Today, 114,000 Abbott employees are working to help people live not just longer, but better, in the more than 160 countries we serve.

As a leading medical technology and nutrition manufacturer, we seek to ensure that CMS policies promote Medicare beneficiary access to high-quality healthcare innovations that address unmet medical needs and improve health outcomes. We therefore commend CMS's intention expressed in the RFI to "reduce barriers to data access and exchange, realize the potential of recent innovations in healthcare that promote better health outcomes, and accelerate progress towards a patient-centric learning health system." Our specific comments follow.

Patients and Caregivers

PC-1. *What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?*

- Continuous Glucose Monitoring (CGM) devices and associated apps help patients monitor their glucose levels continuously, providing real-time data and trends to manage diabetes effectively.
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PC-2. *Do you have easy access to your own and all your loved ones' health information in one location?*

- Integrating CGM data into electronic health record (EHR) systems would allow patients and caregivers to view glucose data alongside other health records, improving overall health management, and would be valuable in contexts such as managing chronic conditions, coordinating care among multiple providers, and during emergency situations.

PC-3. *Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?*

- CGM devices and associated apps are highly beneficial for Medicare beneficiaries managing diabetes since they provide real-time glucose readings, alarms when glucose levels are too high or low, and insights into the individual's health trends, which can be shared with the individual's caregivers.

PC-4 *-- What features are missing from apps you use or that you are aware of today?*

- Patients and their caregivers would benefit from integration of app data with broader health management systems and more comprehensive data analytics tools. We encourage CMS to offer workflows that streamline data sharing between different health systems and providers.

PC-5. *What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?*

- CMS could review and approve digital health products based on efficacy and quality, using criteria such as user-friendliness, data security, and impact on health outcomes.
- Enhancements to data access, reimbursement adjustments, and targeted beneficiary communications also could increase adoption.

PC-6. *What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers?*

- User-friendly interfaces, clear instructions, and robust customer support are crucial for accessibility.

PC-7. *If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?*

- Collecting data through integrated EHR systems reduces administrative burdens, while patient surveys could provide valuable real-world insights.

PC-8. *In your experience, what health data is readily available and valuable to patients or their caregivers or both?*

- Glucose data, medication records, and appointment schedules are readily available and clinically valuable.
- Encounter notes and operative reports are valuable but harder to access.
- Improving interoperability and integration of clinical data from different sources is essential for meaningful clinical research.

PC-9. *Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?*

- Including clinical data, lab results, imaging data, and device data (from CGM devices, for example) would be highly valuable.
- Difficulties include varying data formats and access restrictions.
- Non-CMS data such as patient-generated health data should also be incorporated to the extent possible.

Providers

PR-2. *What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?*

- Generally, such obstacles include interoperability issues, cost, and lack of standardized data formats. Mitigation could involve policy changes and incentives for adopting standardized solutions.

- A specific obstacle relates to virtual performance of services reported with G0248,

  > *demonstration, prior to initiation of home INR monitoring, for patient with either mechanical heart valve(s), chronic atrial fibrillation, or venous thromboembolism who meets Medicare coverage criteria, under the direction of a physician; includes: face-to-face demonstration of use and care of the INR monitor, obtaining at least one blood sample, provision of instructions for reporting home INR test results, and documentation of patient's ability to perform testing and report results*

  Home Prothrombin Time/International Normalized Ratio (IPT/INR) monitoring services are inherently remote services – the patient receives the device from the practitioner for use in the patient's home. The code descriptor, like the national coverage determination (NCD), for G0248 includes the language "face-to-face" when describing the training. In 2025, face-to-face can be achieved by video technology. This mechanism of providing remote training was not available when CMS last revised the NCD in 2008. Technology has evolved such that face-to-face services can be furnished in-person or virtually. Furthermore, CMS has defined the term "face-to-face" to be an in-person or virtual encounter in other circumstances.[1]

  We believe CMS has the authority to permit these services to be furnished in-person or virtually to Medicare patients -- without the addition to the Medicare telehealth list. ***Specifically, we ask that the Agency permit the services described by G0248 to be furnished in-person or virtually in light of the inherently remote nature of home PT/INR monitoring***. CMS has acknowledged that other inherently remote services may be furnished virtually or in-person without requiring such services to be added to the telehealth list.[2] In addition, as noted above, there is no language in the code descriptor or in the NCD itself that requires this service (described by G0248) be furnished *in-person*; rather, the service must only be furnished *face-to-face.*

PR-7. *What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem?*

- We are particularly interested in how CMS uses electronic clinical quality measures (eCQMs) to enhance the healthcare experience in Medicare quality programs. CMS has noted that "[b]y utilizing electronic data captured during the routine process of clinical care, the eCQM has become a critical component of the quality reporting framework."[3] We believe that CMS can use eCGMs to advance its goal of improving the availability of high-quality, timely, and comprehensive data available in the health information ecosystem, while promoting a separate Administration priority of incorporating nutrition as a key component of care quality.[4] Specifically, CMS has adopted the Malnutrition Care Score (MCS) eCQM into the Hospital Inpatient Quality Reporting (IQR) Program. The MCS eCQM assesses adults admitted to inpatient hospital services who received care appropriate to their level of malnutrition risk and malnutrition diagnosis. While hospitals may currently self-select the MCS eCQM to meet the eCQM reporting requirement, we encourage the agency to take the next step to enhance patient nutritional care by making the MCS a mandatory measure for not only the IQR program, but value-based care programs as well.

  Through our work with hospitals and health systems nationwide, we have seen how clinical institutions are already updating their systems and care platforms to collect data and report on

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[1] *See* 42 CFR § 410.38(c)(5).
[2] See 89 Fed. Reg. 97747, which discusses the types of services are not classified as "telehealth services" that involve the use of telecommunications technology to facilitate interactions between the patient and the practitioner (e.g., remote patient monitoring services).
[3] https://mmshub.cms.gov/sites/default/files/eCQM-Specifications-Testing-Standards-Tools-Community.pdf
[4] 90 Fed. Reg. 18,328 (April 30, 2025).

the MCS eCQM, one of the first nutrition focused CMS-approved eCQMs. In addition, the MCS has had a powerful impact in enhancing care coordination and advancing a more nutrition-aware, patient-centered healthcare system. Evidence shows that malnutrition has a direct impact on a hospital's overall quality performance— including influencing readmission rates, pressure injury prevention, infection control, and patient safety events[5,6,7,8,9] —making the appropriate identification and management of patients' nutritional needs a critical strategy for quality that goes beyond the hospital and into post-acute, outpatient, and specialty settings.

As CMS explores future eCQM concepts to improve the health technology ecosystem, we encourage a focus not only on making the MCS mandatory but also on developing new measures that evaluate how well hospitals communicate patients' nutritional status, goals, and care plans across care transitions—particularly into post-acute and ambulatory settings— and ultimately produce improved outcomes. Such measures would advance value-based care by promoting longitudinal, coordinated nutrition management that supports healthier aging and better surgical outcomes, consistent with CMS's broader quality and safety priorities. We urge CMS to lead by integrating the MCS within a comprehensive measurement framework that addresses nutrition care across the full continuum.

Payers

PA-1. *What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?*

- **Policy Limitations:** Current policies may not fully address the integration of CGM data, such as that from FreeStyle Libre, into broader health information exchanges.
- **Technical Limitations:** Interoperability issues and varying data standards can hinder seamless data exchange.
- **Suggestions:** Implementing standardized protocols for CGM data integration and enhancing interoperability standards would address these limitations. These changes would encourage broader participation in TEFCA and improve data sharing.

PA-2. *How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?*

- CMS can provide financial incentives for payers who adopt and utilize APIs for CGM data integration. Additionally, offering technical support and resources for API implementation can accelerate adoption.

PA-3. *How can CMS encourage payers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins?*

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[5] Lee DU, Fan GH, Ahern RR, Karagozian R. The effect of malnutrition on the infectious outcomes of hospitalized patients with cirrhosis: analysis of the 2011-2017 hospital data. *Eur J Gastroenterol Hepatol*. 2021;32(2):269-278. doi:10.1097/MEG.0000000000001991

[6] Chen B, Yang Y, Cai F, et al. Nutritional status as a predictor of the incidence of pressure injury in adults: A systematic review and meta-analysis. *J Tissue Viability*. 2023;32(3):339-348. doi:10.1016/j.jtv.2023.04.005

[7] Guenter P, Abdelhadi R, Anthony P, et al. Malnutrition diagnoses and associated outcomes in hospitalized patients: United States, 2018. *Nutr Clin Pract*. 2021;36(5):957-969. doi:10.1002/ncp.10771

[8] Lengfelder L, Mahlke S, Moore L, Zhang X, Williams G 3rd, Lee J. Prevalence and impact of malnutrition on length of stay, readmission, and discharge destination. *JPEN J Parenter Enteral Nutr*. 2022;46(6):1335-1342. doi:10.1002/jpen.2322

[9] Tappenden KA, Quatrara B, Parkhurst ML, Malone AM, Fanjiang G, Ziegler TR. Critical role of nutrition in improving quality of care: an interdisciplinary call to action to address adult hospital malnutrition. *J Acad Nutr Diet*. 2013;113(9):1219-1237. doi:10.1016/j.jand.2013.05.015

- CMS can mandate the use of standardized digital identity credentials for accessing health information, including CGM data. Providing incentives for payers who adopt these credentials and offering technical support for integration can also encourage acceptance.

PA-4. *What would be the value to payers of a nationwide provider directory that included FHIR endpoints and used digital identity credentials?*

- A nationwide provider directory with FHIR endpoints and digital identity credentials would streamline data exchange, reduce administrative burden, and improve care coordination. These improvements would be particularly valuable for managing chronic conditions like diabetes, where timely access to CGM data is crucial.

PA-7. *How can CMS encourage payers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?*

- CMS can increase awareness about the Information Blocking Portal and provide clear guidelines on how to submit complaints. Encouraging payers to report information blocking practices would advance data exchange by identifying and addressing barriers to interoperability. This would ultimately improve access to comprehensive health information, including CGM data, and enhance patient care.

Technology Vendors, Data Providers, and Networks

TD-2. *Regarding CMS Data, to stimulate developer interest—*

a. *What additional data would be most valuable if made available through CMS APIs?*

- Detailed clinical data, lab results, and patient-generated health data such as CGM data would provide actionable information to improve patient care.

b. *What data sources are most valuable alongside the data available through the Blue Button 2.0 API?*

- Data from EHR systems, pharmacy data, and social determinants of health data would provide a more comprehensive view of patient health.

c. *What obstacles prevent accessing these data sources today?*

- Interoperability issues, varying data formats, and data privacy concerns can hinder access.

d. *What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?*

- We recommend incorporating APIs for real-time health monitoring, predictive analytics, and personalized medicine, specifically incorporating CGM data.

TD-3. *Regarding digital identity implementation:*

a. *What are the challenges and benefits?*

- **Challenges:** User adoption, integration with existing systems, and ensuring data privacy.
- **Benefits:** Improved security, streamlined access to health information, and enhanced patient engagement, including for users of CGM devices.

b. *How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800–63–3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?*

- Digital identity credentials would enhance cybersecurity by ensuring secure access to health information and facilitate seamless data exchange, including CGM data.

c. *What impact would mandatory use of the OpenID Connect identity protocol have?*

- It would standardize identity management, improve interoperability, and reduce the burden of managing multiple credentials, making it easier for patients to access their CGM data.

TD-4. *How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?*

- CMS can mandate the use of open APIs for interoperability and provide incentives for developers who adopt these standards. Offering technical support and resources for implementation can also encourage adoption, which would support our goal of integrating CGM data.

TD-5. *How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?*

- A nationwide provider directory would streamline access to health information, reduce administrative burden, and improve care coordination, including for diabetes management. CMS or a trusted third-party organization should publish the directory, and it should be freely accessible to users.

TD-6. *What unique interoperability functions does TEFCA perform?*

- TEFCA facilitates secure and standardized data exchange across different health information networks, ensuring comprehensive access to health information, including CGM data.

TD-8. *What are the most effective certification criteria and standards under the ONC Health IT Certification Program?*

- Criteria that ensure comprehensive data access, interoperability, and security are most effective. Standards that support real-time data exchange and patient engagement, particularly for CGM data, are also valuable.

TD-9. *Regarding certification of health IT:*

a. *What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?*

- Prioritizing API capabilities would enhance interoperability, streamline data exchange, and support innovation, including to enable integration of CGM data.

b. *What would be the drawbacks?*

- Potential drawbacks include increased complexity in certification processes and the need for ongoing updates to API standards.

c. *How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?*

- Revising criteria to mandate comprehensive API support for all data types would ensure complete data accessibility, including with regard to CGM data, and improve care coordination.

d. *What policy changes could CMS make so providers are motivated to respond to API-based data requests with best possible coverage and quality of data?*

- CMS could provide incentives for high-quality data exchange.

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We appreciate your consideration of our request. Please feel free to contact me if you have any questions or if you need additional information.

Sincerely,

Peter Kelly
Divisional Vice President
Reimbursement and Strategic Initiatives